RTT within 18 weeks

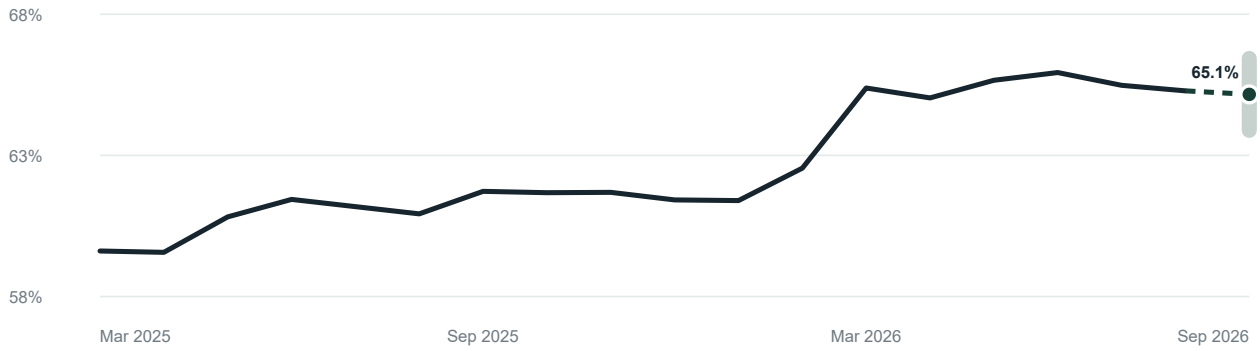
Forecast for September 2026 · data through August 2026

The latest RTT within 18 weeks result was 65.2%. For the next release, it is expected to remain broadly stable at 65.1%. Current-level seasonal and underlying-trend effects are small or offsetting, leaving the outlook broadly stable. The current projection is off course for 70% by March 2027. Across providers, sustained favourable signals outnumber adverse ones (74 to 12).

LATEST · AUGUST 2026	FORECAST · SEPTEMBER 2026	MAR 2027 TARGET OUTLOOK
65.2%	**65.1%**	Projected **66.2%**
0.5pp above forecast	80% expected range 63.9%–66.3%	Target **≥70%**
		7% Model-implied chance

Recent performance and next release

The vertical band is the 80% expected range.



Longer-term outlook and milestones

Descriptive projection; not forced to meet targets.



Benchmark: 92% constitutional standard. 3.8pp below milestone

Data: NHS England referral to treatment waiting times. **Targets:** Medium Term Planning Framework national target · Medium Term Planning Framework final target

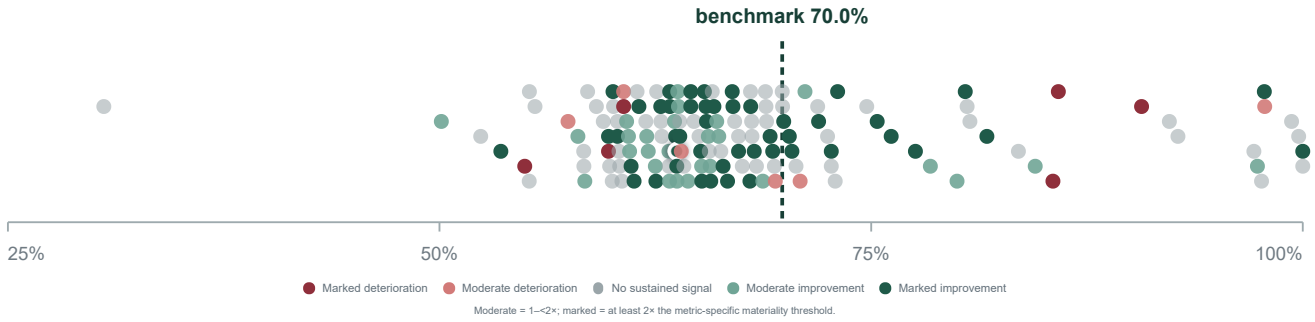
Method: Backtest-weighted ensemble of current-level seasonal movement, damped annual drift and a damped recent seasonal trend; at least 2 components are required. The final fit uses 229 consecutive months. Accuracy is tested on 36 rolling one-month-ahead forecasts. The 80% range uses empirical historical error quantiles from 24 forecast errors. Weights use inverse rolling RMSE and are shrunk toward equal weights. The pre-release commentary compares the current-level seasonal component with the combined forecast. Provider distribution and trajectory watch are shown on page 2.

RTT within 18 weeks

Distribution and trajectory watch · August 2026

Provider distribution

Latest reported performance across providers. This is a comparison of current levels, not a measure of whether each provider is improving or deteriorating.



Moderate = 1–<2×; marked = at least 2× the metric-specific materiality threshold.

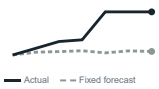
Trajectory watch

Six actual months are compared with one forecast path fixed before that window. Both lines begin at the same observed value. A low performer can be improving, while a high performer can be deteriorating. Status: Actual performance is compared with the monthly path forecast six months earlier. The displayed figure is the latest actual minus the fixed forecast for that same month; the six-month window checks persistence.

139	74	12	53
providers assessed	sustained favourable	sustained adverse	without an active signal

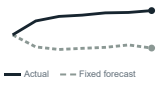
Largest sustained favourable moves

OXLEAS NHS FOUNDATION TRUST
Latest 100.0% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



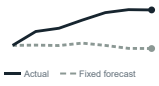
48.1pp
LATEST MONTH: ABOVE FORECAST

COUNTESS OF CHESTER HOSPITAL NHS FOUNDATION TRUST
Latest 68.4% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



15.2pp
LATEST MONTH: ABOVE FORECAST

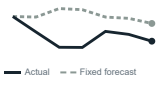
MID CHESHIRE HOSPITALS NHS FOUNDATION TRUST
Latest 70.6% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



14.5pp
LATEST MONTH: ABOVE FORECAST

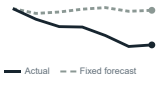
Largest sustained adverse moves

BRADFORD DISTRICT CARE NHS FOUNDATION TRUST
Latest 55.2% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



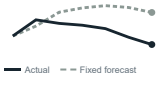
8.4pp
LATEST MONTH: BELOW FORECAST

HAMPSHIRE AND ISLE OF WIGHT HEALTHCARE NHS FOUNDATION TRUST
Latest 85.6% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



6.7pp
LATEST MONTH: BELOW FORECAST

UNIVERSITY HOSPITALS OF NORTH MIDLANDS NHS TRUST
Latest 60.0% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



6.2pp
LATEST MONTH: BELOW FORECAST

Trajectory-watch rule. Six actual months are compared with the corresponding path fixed before the six-month window. The displayed gap is the latest actual minus the fixed forecast for that same month. A signal requires a material latest-month gap and at least five of six months in that direction. These are screening prompts—not league tables, causal estimates or judgements of provider quality.